



ARIZONA HOUSE OF REPRESENTATIVES FLOOR AMENDMENT EXPLANATION

*57th Legislature, 2nd Regular Session
Majority Research Staff*

[HB 2444](#): pharmacists; independent testing; treatment

LOPEZ FLOOR AMENDMENT

1. Clarifies that the statewide written protocol approved by the Pharmacy Board address the listed requirements.
2. Makes additional technical changes.

ADDITIONAL COW
LOPEZ FLOOR AMENDMENT
HOUSE OF REPRESENTATIVES AMENDMENTS TO H.B. 2444
(Reference to House engrossed bill)

Amendment instruction key:

[GREEN UNDERLINING IN BRACKETS] indicates text added to statute or previously enacted session law.
[Green underlining in brackets] indicates text added to new session law or text restoring existing law.
~~[GREEN STRIKEOUT IN BRACKETS]~~ indicates new text removed from statute or previously enacted session law.
~~[Green strikeout in brackets]~~ indicates text removed from existing statute, previously enacted session law or new session law.
<<Green carets>> indicate a section added to the bill.
~~<<Green strikeout in carets>>~~ indicates a section removed from the bill.

1 The bill as proposed to be amended is reprinted as follows:

2 Section 1. Title 32, chapter 18, article 3, Arizona Revised
3 Statutes, is amended by adding sections 32-1979.04, 32-1979.05 and
4 32-1979.06, to read:

5 32-1979.04. Pharmacists; statewide written protocol;
6 independent testing; treatment; health
7 conditions; notification and insurance
8 requirements

9 A. PURSUANT TO A STATEWIDE WRITTEN PROTOCOL APPROVED BY THE BOARD,
10 A PHARMACIST MAY INDEPENDENTLY ORDER, PERFORM AND INTERPRET TESTS THAT ARE
11 AUTHORIZED BY THE UNITED STATES FOOD AND DRUG ADMINISTRATION AND WAIVED
12 UNDER THE CLINICAL LABORATORY IMPROVEMENT AMENDMENTS OF 1988 (P.L.
13 100-578; 102 STAT. 2903; 42 UNITED STATES CODE SECTION 201). A PHARMACIST
14 MAY INDEPENDENTLY INITIATE TREATMENT TO ELIGIBLE PERSONS WHO ARE AT LEAST
15 TWELVE YEARS OF AGE OR THE AGE AUTHORIZED BY THE TREATMENT, WHICHEVER AGE
16 IS OLDER, AND WHO HAVE TEST RESULTS THAT INDICATE THE NEED FOR TREATMENT,
17 BY A TEST THAT IS AUTHORIZED BY THE UNITED STATES FOOD AND DRUG
18 ADMINISTRATION AND WAIVED UNDER THE CLINICAL LABORATORY IMPROVEMENT
19 AMENDMENTS OF 1988, FOR ANY OF THE FOLLOWING:

20 1. INFLUENZA.
21 2. GROUP A STREPTOCOCCUS PHARYNGITIS.
22 3. SARS-COV-2 OR ANY OTHER CORONAVIRUS RESPIRATORY ILLNESS.
23 4. A CONDITION RELATED TO AN EMERGING OR EXISTING PUBLIC HEALTH
24 THREAT IDENTIFIED BY THE DEPARTMENT OF HEALTH SERVICES FOR WHICH A
25 STATEWIDE STANDING ORDER, RULE OR EXECUTIVE ORDER IS ISSUED.

26 B. WHEN DEVELOPING THE STATEWIDE WRITTEN PROTOCOL, THE BOARD SHALL
27 ADDRESS AT A MINIMUM THE FOLLOWING:

- 1 1. DOCUMENTATION.
- 2 2. RECORDS RETENTION.
- 3 3. REFERRALS.
- 4 4. PATIENT SCREENING REQUIREMENTS AND OBTAINING RELEVANT MEDICAL
- 5 HISTORY.
- 6 5. EXCLUSION CRITERIA.
- 7 6. TREATMENT INSTRUCTIONS BASED ON THE PATIENT'S AGE AND MEDICAL
- 8 HISTORY.
- 9 7. FOLLOW-UP MAINTENANCE AND CARE PLANS.
- 10 8. PHARMACIST TRAINING AND CERTIFICATION REQUIREMENTS.
- 11 C. A PHARMACIST WHO ORDERS OR CONDUCTS TESTING OR TREATS HEALTH
- 12 CONDITIONS PURSUANT TO SUBSECTION A OF THIS SECTION SHALL USE ANY TEST
- 13 THAT MAY GUIDE CLINICAL DECISION-MAKING FOR WHICH A WAIVER HAS BEEN
- 14 OBTAINED UNDER THE CLINICAL LABORATORY IMPROVEMENT AMENDMENTS OF 1988, OR
- 15 THE FEDERAL RULES ADOPTED THEREUNDER, OR ANY SCREENING PROCEDURE THAT IS
- 16 ESTABLISHED BY THE STATEWIDE WRITTEN PROTOCOL.
- 17 D. A PHARMACIST SHALL USE EVIDENCE-BASED CLINICAL GUIDELINES
- 18 PUBLISHED BY THE UNITED STATES CENTERS FOR DISEASE CONTROL AND PREVENTION
- 19 OR THE INFECTIOUS DISEASES SOCIETY OF AMERICA, THE AMERICAN ACADEMY OF
- 20 PEDIATRICS COMMITTEE ON INFECTIOUS DISEASE OR ANOTHER CLINICALLY
- 21 RECOGNIZED RECOMMENDATION THAT ALIGNS WITH STANDARDS OF CARE IN PROVIDING
- 22 PATIENT TREATMENT PURSUANT TO SUBSECTION A OF THIS SECTION.
- 23 E. AN ELIGIBLE PERSON MUST MEET CRITERIA FOR TREATMENT BASED ON THE
- 24 STATEWIDE WRITTEN PROTOCOL THAT SPECIFIES THE FOLLOWING:
- 25 1. PATIENT INCLUSION AND EXCLUSION CRITERIA.
- 26 2. EXPLICIT MEDICAL REFERRAL CRITERIA.
- 27 F. A PHARMACIST SHALL REFER A PATIENT TO THE PATIENT'S PRIMARY CARE
- 28 PROVIDER, IF ONE IS IDENTIFIED, OR RECOMMEND FOLLOW UP WITH A PRIMARY CARE
- 29 PROVIDER, IF THE PATIENT EITHER:
- 30 1. IS NOT ELIGIBLE FOR TREATMENT PURSUANT TO THIS SECTION AND
- 31 PRESENTS WITH SYMPTOMS.
- 32 2. DOES NOT RESPOND TO THE INITIAL TREATMENT PROVIDED PURSUANT TO
- 33 THIS SECTION.
- 34 G. A PHARMACIST WHO INITIATES A TREATMENT UNDER THIS SECTION SHALL:
- 35 1. NOTIFY THE PATIENT'S PRIMARY CARE PROVIDER, IF ONE IS
- 36 IDENTIFIED, WITHIN SEVENTY-TWO HOURS AFTER INITIATING TREATMENT PURSUANT
- 37 TO THIS SECTION. THE NOTICE SHALL INCLUDE THE PATIENT'S NAME, THE
- 38 TREATMENT INITIATED AND THE DATE OF TREATMENT AND MAY BE SUBMITTED BY
- 39 ENTRY INTO AN ELECTRONIC HEALTH RECORD OR BY TELEPHONE, FAX, MAIL OR
- 40 EMAIL. THE PHARMACIST SHALL MAKE A REASONABLE EFFORT TO IDENTIFY THE
- 41 PATIENT'S PRIMARY CARE PROVIDER BY AT LEAST ONE OF THE FOLLOWING METHODS:
- 42 (a) CHECKING PHARMACY RECORDS.
- 43 (b) REQUESTING THE INFORMATION FROM THE PATIENT OR, FOR A PATIENT
- 44 UNDER EIGHTEEN YEARS OF AGE, THE PATIENT'S PARENT OR GUARDIAN.
- 45 2. MAINTAIN FOR AT LEAST SEVEN YEARS A RECORD OF THE RESULTS OF ANY
- 46 TESTING OR SCREENING FOR WHICH A TREATMENT IS INITIATED PURSUANT TO THIS

1 SECTION, INCLUDING A SUMMARY OF THE VISIT, PATIENT ASSESSMENT INFORMATION,
2 HISTORY OF ILLNESS, EXAMINATION FINDINGS, VITALS AND PLAN OF CARE.

3 3. NOTIFY THE PATIENT'S PRIMARY CARE PROVIDER, IF ONE IS
4 IDENTIFIED, WITHIN FORTY-EIGHT HOURS AFTER THE OCCURRENCE OF ANY ADVERSE
5 REACTION THAT IS REPORTED TO OR WITNESSED BY THE PHARMACIST AS A RESULT OF
6 THE TREATMENT PROVIDED PURSUANT TO THIS SECTION.

7 4. PROVIDE INFORMATIONAL MATERIALS TO THE PATIENT REQUESTING
8 TREATMENT OR, FOR A PATIENT UNDER EIGHTEEN YEARS OF AGE, TO THE PATIENT'S
9 PARENT OR GUARDIAN ABOUT THE IMPORTANCE OF PEDIATRIC PREVENTIVE HEALTH
10 CARE VISITS AS RECOMMENDED BY THE AMERICAN ACADEMY OF PEDIATRICS.

11 H. A PHARMACIST MAY DELEGATE THE TASK OF PERFORMING A TEST WAIVED
12 BY THE CLINICAL LABORATORY IMPROVEMENT AMENDMENTS OF 1988 TO A LICENSED
13 MEMBER OF THE PHARMACY STAFF WHO IS UNDER THE SUPERVISION OF THE
14 PHARMACIST. A PHARMACIST MAY NOT DELEGATE ANY TASKS THAT INCLUDE CLINICAL
15 JUDGMENT OR TREATMENT AND MAY DELEGATE ONLY ANCILLARY DUTIES AS ALLOWED BY
16 BOARD RULES.

17 I. THIS SECTION DOES NOT REQUIRE A PHARMACIST TO PROVIDE THE
18 SERVICES AUTHORIZED BY THIS SECTION. PURSUANT TO SECTION 32-1901.01,
19 SUBSECTION A, PARAGRAPH 21, AN EMPLOYER MAY NOT OVERRULE A PHARMACIST'S
20 DECISION TO NOT PROVIDE SERVICES THAT ARE AUTHORIZED BY THIS SECTION.

21 J. THIS SECTION DOES NOT ESTABLISH A CAUSE OF ACTION AGAINST A
22 PATIENT'S PRIMARY CARE PROVIDER FOR ANY ADVERSE REACTION, COMPLICATION OR
23 NEGATIVE OUTCOME ARISING FROM ANY TREATMENT INITIATED BY A PHARMACIST
24 PURSUANT TO THIS SECTION.

25 K. A PHARMACIST MAY NOT INDEPENDENTLY INITIATE A TREATMENT USING
26 OPIOIDS FOR A PATIENT.

27 L. A PHARMACIST MAY NOT INDEPENDENTLY ORDER A TEST OR SCREENING OR
28 TREAT A MINOR WITHOUT THE WRITTEN CONSENT OF THE MINOR'S PARENT OR
29 GUARDIAN.

30 M. A PHARMACY SHALL DISPLAY A NOTICE AND INCLUDE IN A PATIENT'S
31 CONSENT PAPERWORK THAT THE TESTING AND TREATMENT BEING PERFORMED PURSUANT
32 TO THIS SECTION ARE BEING PERFORMED BY A PHARMACIST WITHOUT CONSULTATION
33 WITH OR OVERSIGHT BY A PHYSICIAN AND THAT THE PATIENT SHOULD CONSULT WITH
34 A PRIMARY CARE PROVIDER IF SYMPTOMS CONTINUE.

35 N. A PHARMACIST SHALL MAINTAIN PROFESSIONAL LIABILITY INSURANCE IF
36 PERFORMING SERVICES THAT ARE AUTHORIZED UNDER THIS SECTION.

37 32-1979.05. Independent testing and treatment advisory
38 committee; duties; members

39 A. THE BOARD SHALL ESTABLISH THE INDEPENDENT TESTING AND TREATMENT
40 ADVISORY COMMITTEE TO ASSIST THE BOARD IN DEVELOPING THIS STATE'S WRITTEN
41 PROTOCOLS RELATING TO PHARMACISTS' INDEPENDENT AUTHORITY TO ORDER TESTING
42 AND INITIATE TREATMENTS PURSUANT TO ~~[SECTION]~~ ~~[SECTIONS]~~ 32-1979.04 ~~[AND~~
43 ~~32-1979.06]~~. THE ADVISORY COMMITTEE SHALL ALSO MAKE RECOMMENDATIONS TO
44 THE BOARD REGARDING THE PROTOCOLS REQUIRED PURSUANT TO THAT SECTION.

45 B. THE ADVISORY COMMITTEE SHALL INCLUDE AT LEAST THE FOLLOWING:

46 1. TWO PHARMACISTS WHO ARE LICENSED PURSUANT TO THIS CHAPTER AND
47 WHO ARE APPOINTED BY THE BOARD.

1 2. TWO PHYSICIANS WHO ARE LICENSED PURSUANT TO THIS TITLE, ONE WHO
2 SPECIALIZES IN PRIMARY CARE AND ONE WHO PRACTICES IN AN EMERGENCY MEDICINE
3 SETTING, AND WHO ARE APPOINTED BY THE ARIZONA MEDICAL BOARD OR THE ARIZONA
4 BOARD OF OSTEOPATHIC EXAMINERS IN MEDICINE AND SURGERY. AT LEAST ONE OF
5 THESE MEMBERS MUST HAVE A PATIENT POPULATION THAT IS SUBSTANTIALLY
6 COMPOSED OF CHILDREN AND ADOLESCENTS.

7 3. ONE PERSON WHO REPRESENTS A NONPROFIT PATIENT ADVOCACY
8 ORGANIZATION AND WHO IS APPOINTED BY THE GOVERNOR.

9 4. ONE NURSE PRACTITIONER WHO IS LICENSED PURSUANT TO CHAPTER 15 OF
10 THIS TITLE, WHO SPECIALIZES IN PRIMARY CARE OR EMERGENCY MEDICINE, WHO IS
11 ABLE TO PRESCRIBE MEDICATION AND WHO IS APPOINTED BY THE ARIZONA STATE
12 BOARD OF NURSING.

13 C. ADVISORY COMMITTEE MEMBERS ARE NOT ELIGIBLE FOR COMPENSATION OR
14 REIMBURSEMENT OF EXPENSES.

15 D. THE ADVISORY COMMITTEE SHALL REVIEW THE STATEWIDE PROTOCOLS
16 ANNUALLY AND UPDATE WHEN NECESSARY.

17 32-1979.06. Human immunodeficiency virus prevention;
18 preexposure prophylaxis; postexposure
19 prophylaxis; definitions

20 A. IN ADDITION TO THE AUTHORITY PROVIDED IN SECTION 32-1979.04, A
21 PHARMACIST WHO IS LICENSED PURSUANT TO THIS CHAPTER MAY INITIATE AND
22 DISPENSE HUMAN IMMUNODEFICIENCY VIRUS PREEXPOSURE PROPHYLAXIS AND
23 POSTEXPOSURE PROPHYLAXIS PURSUANT TO THE STATEWIDE WRITTEN PROTOCOL
24 ~~[ADOPTED]~~ [APPROVED] BY THE BOARD.

25 B. BEFORE INITIATING A PREEXPOSURE PROPHYLAXIS, A PHARMACIST SHALL:

26 1. CONDUCT OR OBTAIN A CLINICAL LABORATORY IMPROVEMENT
27 AMENDMENTS-WAIVED HUMAN IMMUNODEFICIENCY VIRUS TEST AND CONFIRM A NEGATIVE
28 TEST RESULT.

29 2. ASSESS THE PATIENT FOR CONTRAINDICATIONS, DRUG INTERACTIONS AND
30 CLINICAL ELIGIBILITY CONSISTENT WITH THE CURRENT GUIDELINES OF THE UNITED
31 STATES CENTERS FOR DISEASE CONTROL AND PREVENTION.

32 C. BEFORE INITIATING A POSTEXPOSURE PROPHYLAXIS, A PHARMACIST
33 SHALL:

34 1. DETERMINE THAT THE PATIENT PRESENTS WITHIN SEVENTY-TWO HOURS
35 AFTER A POTENTIAL EXPOSURE TO THE HUMAN IMMUNODEFICIENCY VIRUS.

36 2. CONDUCT OR OBTAIN A HUMAN IMMUNODEFICIENCY VIRUS TEST IF
37 FEASIBLE UNDER THE STATEWIDE WRITTEN PROTOCOL.

38 3. ASSESS THE PATIENT FOR CONTRAINDICATIONS, DRUG INTERACTIONS AND
39 CLINICAL ELIGIBILITY CONSISTENT WITH THE CURRENT GUIDELINES OF THE UNITED
40 STATES CENTERS FOR DISEASE CONTROL AND PREVENTION.

41 D. A PHARMACIST WHO INITIATES THERAPY PURSUANT TO THIS SECTION:

42 1. MAY DISPENSE NOT MORE THAN A THIRTY-DAY SUPPLY OF PREEXPOSURE
43 PROPHYLAXIS.

44 2. MAY DISPENSE A COMPLETE TWENTY-EIGHT-DAY COURSE OF POSTEXPOSURE
45 PROPHYLAXIS.

46 3. SHALL PROVIDE COUNSELING REGARDING MEDICATION ADHERENCE,
47 POTENTIAL SIDE EFFECTS AND PREVENTION MEASURES.

1 4. SHALL PROVIDE THE PATIENT WITH A WRITTEN REFERRAL TO A PRIMARY
2 CARE PROVIDER OR OTHER APPROPRIATE HEALTH CARE PROVIDER FOR FOLLOW-UP
3 CARE.

4 5. SHALL NOTIFY THE PATIENT'S PRIMARY CARE PROVIDER OF THE SERVICES
5 PROVIDED IF THE PATIENT CONSENTS TO THE NOTIFICATION.

6 E. THE ~~[BOARD SHALL ADOPT RULES NECESSARY TO IMPLEMENT THIS~~
7 ~~SECTION, INCLUDING]~~ [STATEWIDE WRITTEN PROTOCOL APPROVED BY THE BOARD
8 SHALL ADDRESS, AT A MINIMUM, THE FOLLOWING]:

9 1. MINIMUM TRAINING REQUIREMENTS FOR PHARMACISTS WHO INITIATE
10 THERAPY PURSUANT TO THIS SECTION.

11 2. DOCUMENTATION AND RECORDS RETENTION REQUIREMENTS.

12 3. STATEWIDE WRITTEN PROTOCOL STANDARDS THAT ARE CONSISTENT WITH
13 THE CURRENT GUIDELINES OF THE UNITED STATES CENTERS FOR DISEASE CONTROL
14 AND PREVENTION OR THE INFECTIOUS DISEASES SOCIETY OF AMERICA OR ANOTHER
15 CLINICALLY RECOGNIZED RECOMMENDATION.

16 F. FOR THE PURPOSES OF THIS SECTION:

17 1. "POSTEXPOSURE PROPHYLAXIS" MEANS A MEDICATION REGIMEN THAT IS
18 APPROVED BY THE UNITED STATES FOOD AND DRUG ADMINISTRATION AND THAT IS
19 INITIATED AFTER POTENTIAL EXPOSURE TO THE HUMAN IMMUNODEFICIENCY VIRUS TO
20 PREVENT INFECTION.

21 2. "PREEXPOSURE PROPHYLAXIS" MEANS A MEDICATION REGIMEN THAT IS
22 APPROVED BY THE UNITED STATES FOOD AND DRUG ADMINISTRATION TO PREVENT
23 HUMAN IMMUNODEFICIENCY VIRUS INFECTION IN PERSONS WHO ARE AT RISK.

24 Enroll and engross to conform

25 Amend title to conform

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